



Armada Kalamunda Group

ED Surge and Escalation Plan Guideline

1. Purpose

This guideline sets out an escalation plan to assist with surge management within the emergency department of Armada Health Service. Escalation relating to access block will follow the Bed Capacity Management Plan. The ED surge management plan outlines an overview of influencing factors that impact ED capacity

2. Guidelines

As the emergency department condition worsens, the degree of response heightens through activation of escalation codes. The escalation proceeds in 4 levels which are designated by colour – **Green** BAU (Business as Usual), **Amber**, **Red** and finally **Black**.

When triggers for **red** - escalation within the ED leadership team- nursing and medical. When **black** status is achieved the shift coordinator for ED must inform the Hospital Nurse Manager as outlined in the chart below.

2.1 ED escalation grid:

Escalate if two or more criteria occur.

Status	Staffing	Occupancy	Acuity	Access Block
Green	At set profile for medical and nursing staffing	>= 40 patients	ATS 1: in time ATS 2: in time	Hospital Beds available
Amber	Medical/ nursing staffing below profile	> 40 patients	ATS 1:1 pt. in previous hour ATS 2:<10mins ANUM/Coordinator escalates concerns re: acuity in department	Patients waiting > 4 hours or > 4 patients waiting for a bed or 1 ambulance ramping
Red	Medical/ nursing staffing below profile (can include escort)	> 50 patients	ATS 1:> 1 pt. ATS 2:> 4 in previous hour ATS 3:> 6 in previous hour ANUM/Coordinator escalates concerns re: acuity in department	Patients waiting > 6 hours or > 6 patients waiting for a bed or 2 ambulances ramping

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Status	Staffing	Occupancy	Acuity	Access Block
Black	Medical/ nursing staffing below profile 2 or more	> 60 patients	ATS 1:> 2 pt. in previous hour ATS 2:> 6 in previous hour ATS 3:> 8 in previous hour ANUM/Coordinator escalates concerns re: acuity in department	Patients waiting > 8 hours or > 8 patients waiting for a bed or 3 or more ambulances ramping

3. Roles and responsibilities

Status	Responsible Person	Action	Outcome
Green	ED Coordinator	Monitors activity 30 minutely	Maintain patient flow
	ED Duty Officer	Clinical rounding	Maintain patient flow
Amber	ED Coordinator	Discuss with Team leaders- identify patients for discharge Identify patients to decant to corridor space Review ambulance ramped patient against patient in WR to identify greatest need for the next available bay Inform Hospital Nurse Manager change in Status	Ensure access to bays for higher acuity patients. Facilitate and prompt timely discharges Utilise next available bay for the patient who is deemed highest priority Elevates ED referrals in bed management priority
	ED Duty Officer	Clinical rounding with consultants and registrars with focus on early plans and discharge disposition	Improved and timely decision making Support for junior staff Increased visibility on patient plans

Status	Responsible Person	Action	Outcome
Red	ED Coordinator	Identify patients of concerns and escalate to the DO	Maintain patient safety and appropriate allocation of resources
		Inform Hospital Nurse Manager change in Status	All admissions to hospital beds will be from the ED as a priority. Delays IHT's in. Review ability to source additional staff to support ongoing care
		Review ambulance ramped patient against patient in WR to identify greatest need for the next available bay	Utilise next available bay for the patient who is deemed highest priority
	ED Duty Officer	Review all patients in high acuity areas.	Problem solving delays. Early bed request and referral to specialities within and external to the hospital
	ANUM/ AH CNS	Review POC/ deterioration/ patients waiting for admission/ transfer. Allocate non-clinical resources from within the department to support patient care	Facilitate and support the ED Coordinator to maintain clearly documented and timely plan and ability to provide safe patient care
Black	ED Coordinator	Identify patients who can be allocated to corridor spaces and the waiting room in consultation with the DO. Ensure that resources provided are allocated to the area of highest need.	Maintain patient safety and appropriate allocation of resources
		Review ambulance ramped patient against patient in WR to identify greatest need for the next available bay	Utilise next available bay for the patient who is deemed highest priority
		Inform Hospital Nurse Manager change in Status	

Status	Responsible Person	Action	Outcome
			Cancels all IHT's into hospital, all admissions ED priority.
	ED Duty Officer	Reviews the waiting room presentations and considers allocation of medical resources. Escalate to the HOD as required Considers the request for bypass through the Hospital Nurse Manager (HNM) to Senior Responder on Call (SROC)	Early commencement of care in the WR, prompt decision making
	ANUM/ AH CNS	Allocate non-clinical resources from outside the department to support patient care. ANUM will assume Coordinator role.	Consideration of requesting permission to offer overtime
	HNM	Support and escalation Essendex, Nurse West calls, overtime discussions to increase staffing as needed	Contact SROC as required and appropriate Safe staffing
	SROC	Review current ambulance status across health and discuss bypass ability with ADC	Reduce ambulance presentations and ramping in the department
		Safe staffing	Approve overtime request
		Consider overs census beds across the hospital if access block is a contributing factor	Reduce the number of patients in ED and facilitate assessments in bays and off loading ramped ambulances

4. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.30: Identifies service areas that have a high risk of unpredictable behaviours and develop strategies to minimise risk if harm for patients, carers, families and the workforce.
- Standard 3 – Action 3.11: respond to environmental risks.

- Standard 5 – Action 5.04: provide care to patients in the setting best meets their clinical needs.
- Standard 5 – Action 5.05: Support multidisciplinary collaboration and teamwork.
- Standard 6 – Action 6.03: Actively involve patients in their own care.
- Standard 6 – Action 6.09: Communicating critical information.
- Standard 8 – Action 8.04: Monitor patient as required by their individualised monitoring plan.
- Standard 8 – Action 8.05: Monitor patients at risk of acute deterioration in mental state, including patients at risk of developing delirium.
- Standard 8 – Action 8.06: Escalation of care
- National Standards for Mental Health Services (2010):
- Standard 1 – Criteria 1.1: The MHS always upholds the right of the consumer to be treated with respect and dignity.
- Standard 2 – Criteria 2.6: The MHS meets their legal occupational health and safety obligations to provide a safe workplace and environment.

5. Acknowledgements

[Patient Flow Escalation Process](#)

South Metropolitan Area Health Service

6. Glossary

The following definitions are relevant to this policy.

Term	Definition
Surge	A sudden increase in patient numbers above normal expected amounts, of increasing acuity, over a period of 1 hour.
Escalation	The next level of intervention, higher grade of seniority of staff involvement, wider level of hospital involvement to implement change
NUM	Nurse Unit Manager
ANUM	Associate Nurse Unit Manager
IHT	Inter Hospital Transfer
HNM	Hospital Nurse Manager
ATS	Australian Triage Score
HOD	Head of Department
ED	Emergency Department
WR	Waiting Room

7. Document contact

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8. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V1.0	11/04/2023	11/04/2026	-

9. Authorisation

This guideline has been authorised and issued by the Director of Nursing and Midwifery.

Authorised by	Helen Fullarton -- Director of Nursing and Midwifery
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